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Postpartum depression

KEY POINTS

Postpartum depression (also called PPD) affects about 1 in 7 women (around 14%) in the United States.

PPD can make caring for yourself and your baby harder. It's a serious but treatable condition. ...

Find support

Ways to give

Our work

... **About us** ... provider right away.

PPD is not your fault, and it doesn't make you a bad parent. Getting care early matters for you and your baby.

Download our English and Spanish health action sheets

<<https://www.marchofdimes.org/sites/default/files/2026-03/mod-ppd-health-action-sheet-updated-2026.pdf>> on postpartum depression.

Download more information on postpartum depression.

<https://www.marchofdimes.org/sites/default/files/2023-07/mod_postpartumdepression_onepager_v09_final.pdf>

What is postpartum depression?

Postpartum depression (also called PPD) is a type of depression that begins after birth and involves strong feelings of sadness, anxiety (worry) and tiredness (fatigue). Although it often starts 1 to 3 weeks after giving birth, PPD can begin anytime in the first year.

Is PPD the same as the baby blues?

No, PPD is not the same as the "baby blues". It lasts longer and it's a more serious condition that needs treatment to get better.

Baby blues <<https://www.marchofdimes.org/find-support/topics/postpartum/baby-blues-after-pregnancy>> are feelings of sadness you may have after having a baby. It can happen 2 to 3 days after giving birth and can last up to 2 weeks. Signs and

symptoms of the baby blues include having trouble sleeping, being moody or cranky, and crying a lot. The baby blues are very common but if you have sad feelings that last longer than 2 weeks, tell your provider.

What are the signs and symptoms of PPD?

You may have PPD if you have five or more signs or symptoms that last longer than 2 weeks. Signs and symptoms of PPD include:

Changes in your feelings:

- Feeling depressed most of the day every day
- Feeling shame, guilt or like a failure
- Feeling panicked or scared a lot of the time
- Having severe mood swings

Changes in your everyday life:

- Having little interest in things you normally like to do
- Feeling tired all the time
- Eating a lot more or a lot less than is normal for you
- Gaining or losing weight
- Having trouble sleeping or sleeping too much
- Having trouble concentrating or making decisions

Changes in how you think about yourself or your baby:

1. Having trouble bonding with your baby
2. Thinking about hurting yourself or your baby
3. Thinking about suicide (killing yourself)

If you think you have signs or symptoms of PPD, call your healthcare provider right away. There are things you and your provider can do to help you feel better. If you're worried about hurting yourself or your baby, call emergency services at 911.

Can PPD affect your baby?

Yes. PPD can make it hard for you to care for yourself and your baby. Untreated PPD can affect:

- How you take care of yourself. For example, missing postpartum checkups <<https://www.marchofdimes.org/find-support/topics/postpartum/your-postpartum-checkups>>.
- Bonding with your baby.
- The time you spend breastfeeding. PPD may make it hard for you and your baby to get used to breastfeeding <<https://www.marchofdimes.org/find-support/topics/parenthood/breastfeeding-your-baby>>.
- How you take care of your baby. You may not see health problems in your baby that need quick attention and care. Or, it may be hard for you to get your baby regular well-baby care <<https://www.marchofdimes.org/find-support/topics/parenthood/your-babys-checkups>>, like vaccinations <<https://www.marchofdimes.org/find-support/topics/birth/your-babys-vaccinations>>.
- Your baby's development. This could lead to learning, behavior and development problems and mental health conditions later in life.

Getting treatment for PPD can help you feel better and be able to care for your baby. If you think you have PPD, tell your provider.

What causes PPD?

We're not exactly sure what causes PPD, but we do know that many factors play a role. This includes:

- Genes. Depression is more common in people with a family history <<https://www.marchofdimes.org/find-support/topics/planning-baby/your-family-health-history>> of depression.
- Changing hormone levels after pregnancy. During pregnancy, your body has higher levels of the hormones estrogen and progesterone. But in the first 24 hours after giving birth, these hormones quickly go back to their normal levels. This quick drop in hormone levels may lead to PPD.

- Low levels of thyroid hormones. The thyroid is a small gland in your neck that helps control how your body uses energy. When it doesn't make enough hormones, it can cause symptoms like tiredness, mood changes, and trouble sleeping— all of which can increase your chances of having postpartum depression.

Are you at risk for PPD?

Some things may raise your chances of having postpartum depression (also called PPD). These are called risk factors. Having one or more doesn't mean you'll have PPD, but it's important to know what they are. Talk to your healthcare provider about your risk— asking questions is a strong first step.

You can also ask your provider to screen you for PPD during your postpartum checkups. During your checkup, you'll be asked a few questions about how you're feeling. If the results show you may be at risk or already have symptoms, your provider can connect you with the right care and support.

Risk factors for PPD include:

- Depression during pregnancy, or you've had major depression or another mental health condition in the past. You may also have a family history of these conditions.
- You've experienced physical or sexual abuse. Or you're in a relationship with problems, including domestic violence <<https://www.marchofdimes.org/find-support/topics/pregnancy/abuse-during-pregnancy>> (also called intimate partner violence or IPV).
- You're dealing with high stress. This could be from losing a loved one, being separated from a partner, having a serious illness, or facing financial or housing challenges. It can also include having limited support, being a young parent, or navigating a pregnancy that you didn't plan for.
- You have diabetes. This includes diabetes before pregnancy (called pregestational) or diabetes that starts during pregnancy (gestational diabetes).

- You have complications during pregnancy, like preterm birth <<https://www.marchofdimes.org/find-support/topics/birth/preterm-labor-and-preterm-birth-are-you-risk>>, being pregnant with multiples <<https://www.marchofdimes.org/find-support/topics/pregnancy/being-pregnant-twins-triplets-and-other-multiples>>, having a baby with birth defects <<https://www.marchofdimes.org/find-support/topics/planning-baby/birth-defects-and-your-baby>> or experiencing pregnancy loss <<https://www.marchofdimes.org/find-support/topics/miscarriage-loss-and-grief>>.
- You smoke <<https://www.marchofdimes.org/find-support/topics/pregnancy/smoking-during-pregnancy>>, drink alcohol <<https://www.marchofdimes.org/find-support/topics/pregnancy/alcohol-during-pregnancy>> or take harmful drugs <<https://www.marchofdimes.org/find-support/topics/pregnancy/street-drugs-and-pregnancy>>.
- You're having a hard time breastfeeding or taking care of your baby. Maybe your baby has health problems, or you're struggling with thoughts or feelings about being a parent.

Negative thoughts and feelings about being a mom can include:

- Doubting whether you're a good mom
- Feeling pressure to be a perfect parent
- Feeling like you've lost a part of who you were before having your baby
- Feeling less attractive since giving birth
- Having no free time for yourself
- Feeling tired and moody because you not sleeping well or getting enough sleep

You're not alone in feeling this way. Don't be afraid to talk to your provider or ask for help.

Can PPD be prevented?

Some women are more likely to get PPD than others. The good news is that certain kinds of counseling (also called therapy) can help prevent it for women at increased risk.

Counseling gives you a safe space to talk about how you're feeling and learn how to manage stress, emotions, or difficult life changes. A trained counselor or therapist can help you better understand your thoughts and feelings and guide

you on mindfulness exercises that can help you solve problems and cope with things in your everyday life.

According to teams of experts, such as the U.S. Preventive Services Task Force, counseling may help if you have one or more of these risk factors for PPD:

- You're already feeling symptoms of depression
- You've had depression or another mental health condition in the past
- You're a single parent or became pregnant as a teen
- You're going through a tough time. For example, you have low income or little support
- You've experienced abuse or violence with your partner

There are two common types of counseling that may help prevent PPD:

1. **Cognitive behavioral therapy (also called CBT).** CBT helps you manage negative thoughts by changing the way you think and act in situations.
2. **Interpersonal therapy (also called IPT).** IPT focuses on relationships and major life events. It helps you deal with challenges you might have with your partner, family, or work.

If you're at higher risk for PPD, talk to your provider about CBT and IPT. Being open and honest about your experiences can help your provider connect you with the right support and treatment.

How is PPD treated?

If you think you might have PPD, don't wait—talk with your healthcare provider as soon as you can. Your provider can be:

- Your OB-GYN or prenatal care provider.
- Your regular doctor or primary care provider.
- A mental health professional, like a therapist, counselor, psychologist, or psychiatrist.
- Your baby's healthcare provider, who can also connect you to care.

Your provider may ask you questions about how you're feeling and if your feelings are causing problems in how you care for yourself and your baby. You may be asked to fill out a form called a depression screening questionnaire. Your answers on the form can help him find out if you have PPD. Your provider may also check to see if you have other health problems that may lead to PPD. This can include a check of your thyroid levels hormones.

The sooner you see your provider about PPD, the better. Treatment can include:

- Talk therapy, like CBT and IPT
- Support groups, where you can connect with others going through similar experience.
- Prescription medicine to help manage your symptoms.

Medicines to treat PPD include:

- **Antidepressants.** These are medicines used to treat different kinds of depression, including PPD. Some have side effects and not all are safe to take if you're breastfeeding <<https://www.marchofdimes.org/find-support/topics/parenthood/keeping-breast-milk-safe-and-healthy>>. Talk to your provider to find out about what's right for you.
- **Estrogen therapy.** This hormone plays an important role in your menstrual cycle and pregnancy. During childbirth, the amount of estrogen in your body drops quickly. If your provider thinks your hormone levels are playing a role, estrogen therapy may be recommended to you. Be sure to ask if it's safe while breastfeeding.

A few important reminders:

- Don't start or stop taking any medicines for PPD without your provider's OK.
- Take your medication exactly as prescribed.
- If you're breastfeeding, talk to your provider to make sure what you're taking is safe for your baby.
- Some herbal supplements, like St. John's wort, are not proven safe for PPD. Avoid using them unless your provider says it's OK.

What else can help?

Treatment works best when you take care of your whole self. Here's what can help:

- **Move your body and eat whole foods.** Go for a walk or do an activity you enjoy. Choose fruits, vegetables, whole-grains and lean protein as much as possible.
- **Get as much rest as you can.** Nap when your baby sleeps
<<https://www.marchofdimes.org/find-support/topics/parenthood/safe-sleep-your-baby>> whenever possible.
- **Avoid alcohol and drugs.** They can worsen depression and interfere with your treatment. Alcohol is a depressant, which means it can slow your body down and make you feel more depressed. You also can pass alcohol and drugs to your baby through breast milk.
- **Ask for help and talk it out.** Whether it's with baby care, meals, or chores, let loved ones support you. Tell someone you trust how you're feeling. This can include your partner, a friend, or family member.
- **Take breaks.** Spend time outside, do something creative, or reconnect with things you enjoyed before having your baby.
- **Ease back into routines.** Avoid big life changes if you can, and talk to your employer about flexible work options.

What is postpartum psychosis?

Postpartum psychosis is a rare, severe form of depression. It's not the same as PPD and needs immediate care. It usually begins in the first 2 weeks after birth. You may be at higher risk if you have bipolar disorder or a similar mental health condition. Call your healthcare provider right away if you have any of these signs or symptoms:

- Seeing or hearing things that aren't there
- Feeling confused, paranoid, or very upset
- Having rapid mood swings or changes
- Trouble sleeping, even when you're really tired
- Feeling hopeless, upset or restless
- Thinking about or trying to hurt yourself, your baby, or others

If you notice any of these signs, or if you're worried you might hurt yourself or your baby, call 911 or emergency services right away. You're not alone and there is help available.

More support

- Postpartum Support International <<http://www.postpartum.net/>>, 800-944-4PPD (4773)
- National Maternal Mental Health Hotline <<https://mchb.hrsa.gov/programs-impact/national-maternal-mental-health-hotline>> 1-833-TLC-MAMA (852-6262)
- National Suicide Prevention Lifeline <<https://suicidepreventionlifeline.org/>>, 800-273-TALK (8255)
- National Alliance on Mental Illness <<http://nami.org/>>, 800-950-NAMI (6264)
- Depression during and after pregnancy: A resource for women, their families and friends
<https://mchb.hrsa.gov/sites/default/files/mchb/maternalchildhealthtopics/maternal-womens-health/depression_during_and_after_pregnancy_english.pdf> from the U.S. Department of Health and Human Services, Health Resources and Services Administration, Office of Maternal and Child Health
- Mothertobaby.org <<http://www.mothertobaby.org/>>, information on medications and more

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